

HISTOPATHOLOGICAL EVALUATION OF ENDOMETRIAL BIOPSIES AND CURETTINGS IN DYSFUNCTIONAL UTERINE BLEEDING

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INTRODUCTION

Abnormal uterine bleeding (AUB) is an umbrella term encompassing various forms of uterine bleeding that deviate from the normal menstrual pattern in terms of regularity, volume, duration, or frequency

THE PALM-COEIN SYSTEM

- P** - Polyp (AUB-P)
- A** - Adenomyosis (AUB-A)
- L** - Leiomyoma (AUB-L)
- M** - Malignancy and Hyperplasia (AUB- M)
- C** - Coagulopathy (AUB-C)
- O** - Ovulatory Dysfunction (AUB-O)
- E** - Endometrial (AUB-E)
- I** - Iatrogenic (AUB-I)
- N** - Iatrogenic and Not Otherwise Classified (AUB-N)

AIMS & OBJECTIVES

- To identify the various histopathological patterns in the endometrial biopsies and curettings in the patients of AUB
- To see the correlation of various histological patterns with age and clinical profiles including LMP, drug history and radiological findings(if any) in these patients.

METHODOLOGY

- **Inclusion Criteria:** Endometrial biopsies and curettings from the patients presenting with AUB irrespective of age group (excusing unmarried girls)
- **Exclusion Criteria:** Patients presented with unsatisfactory samples like only blood clots & fibrin and no endometrial glands/stroma.
 - AUB due to gestational causes like tubal pregnancy, molar pregnancy and abortion Patients on hormone therapy within the last 6 months.
 - Cervical pathology e.g cervical cancer.
- **Study Design:** One year retrospective and 2 year prospective (2022-2025)
- **Sample Size:** 351 patients

RESULTS

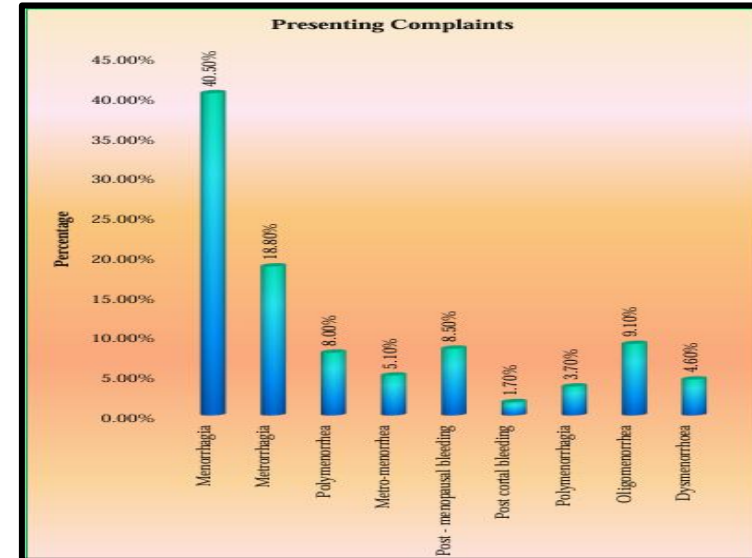
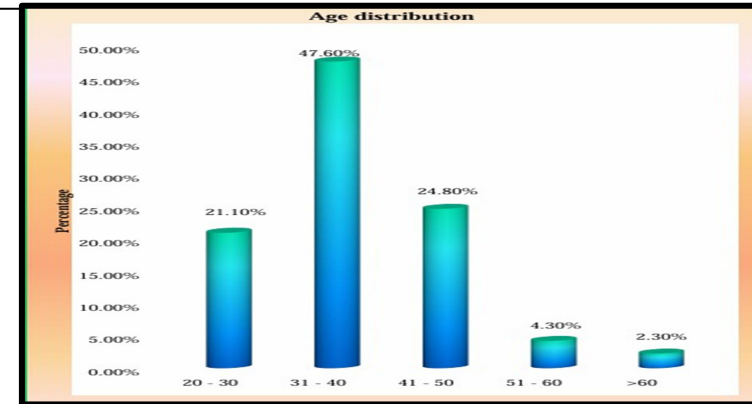
Age Distribution:

- Most common age group 31-40y
- Least Common age group >60y

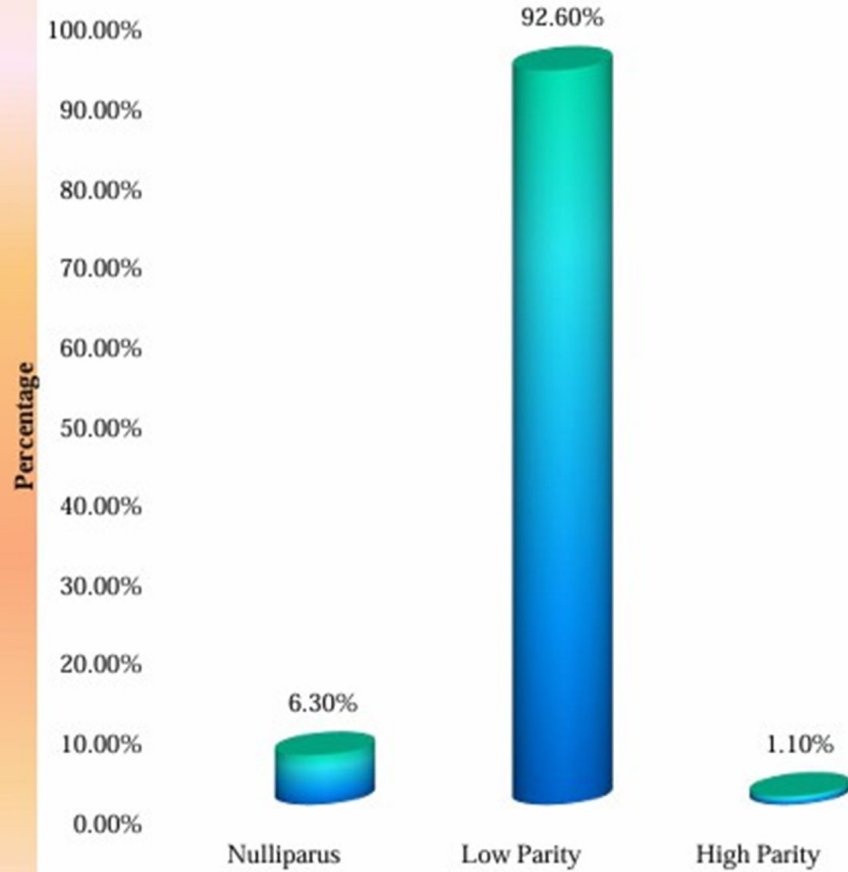
PRESENTING COMPLAINT

Menorrhagia emerged as the most common presenting complaint at 40.5%, highlighting the prevalence of heavy bleeding patterns that prompt medical evaluation.

- **Metrorrhagia: 18.8%**
- **Polymenorrhea: 18.8%**
- **Post-Menopausal Bleeding: 8.50%**

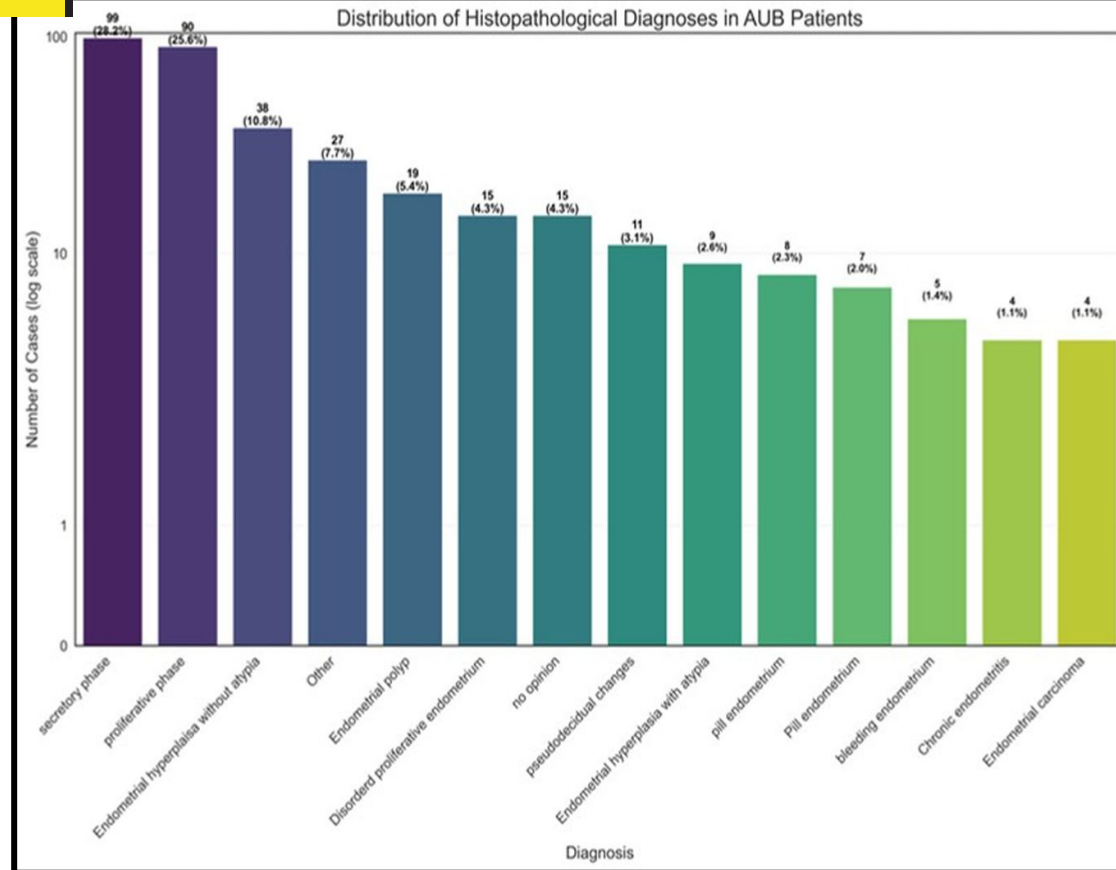


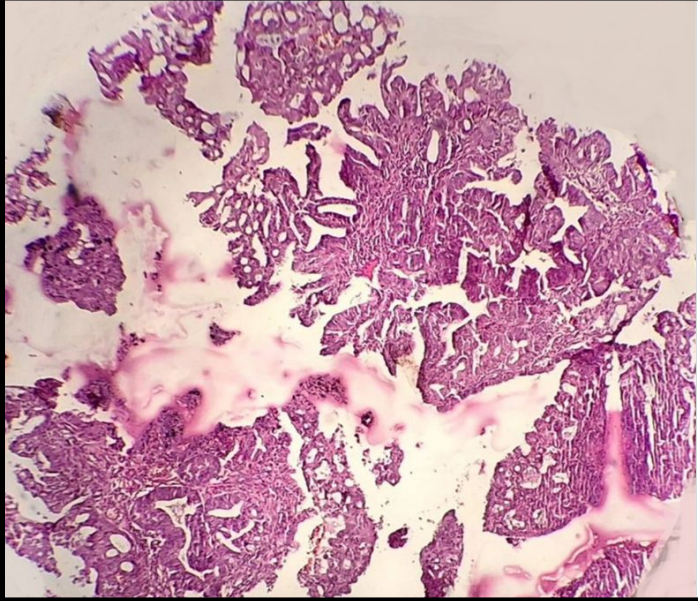
Parity



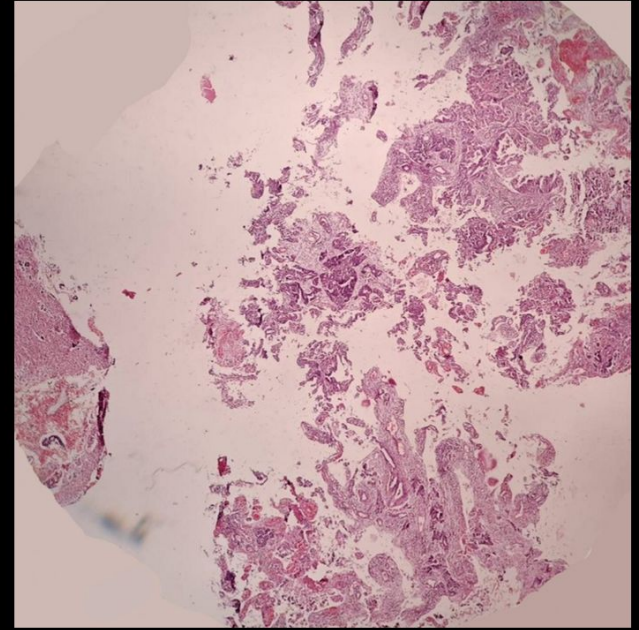
Histopathological pattern

- Secretory Phase: 29.3%
- Proliferative Phase: 25.6%
- Atypical Hyperplasia 2.8%

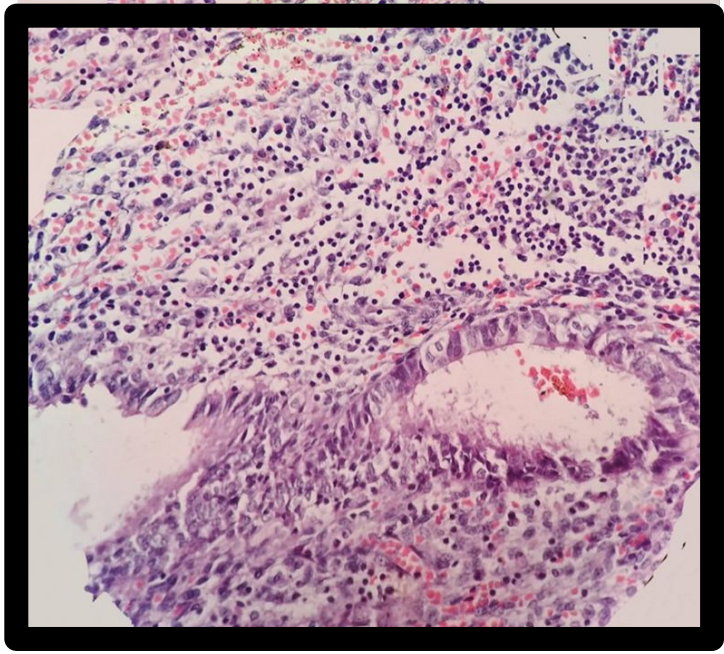




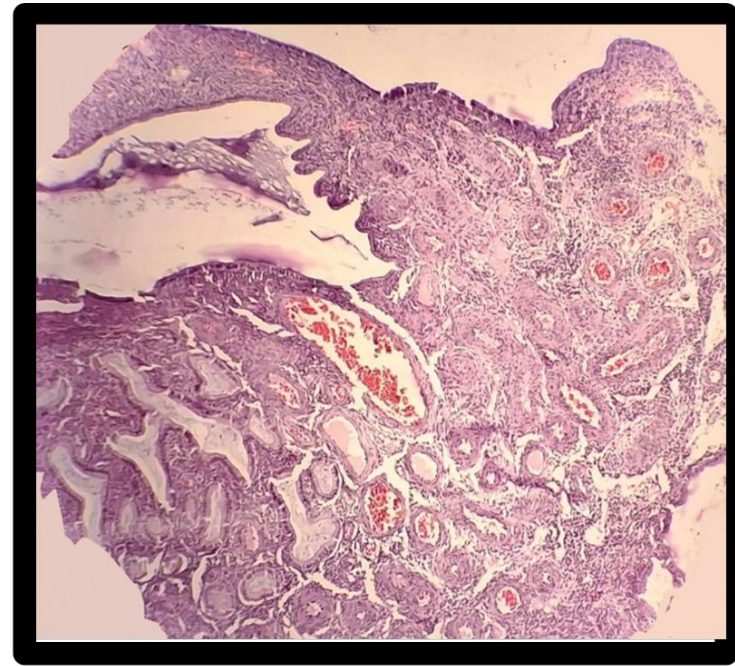
**Cribriform pattern
endometrioid
carcinoma**



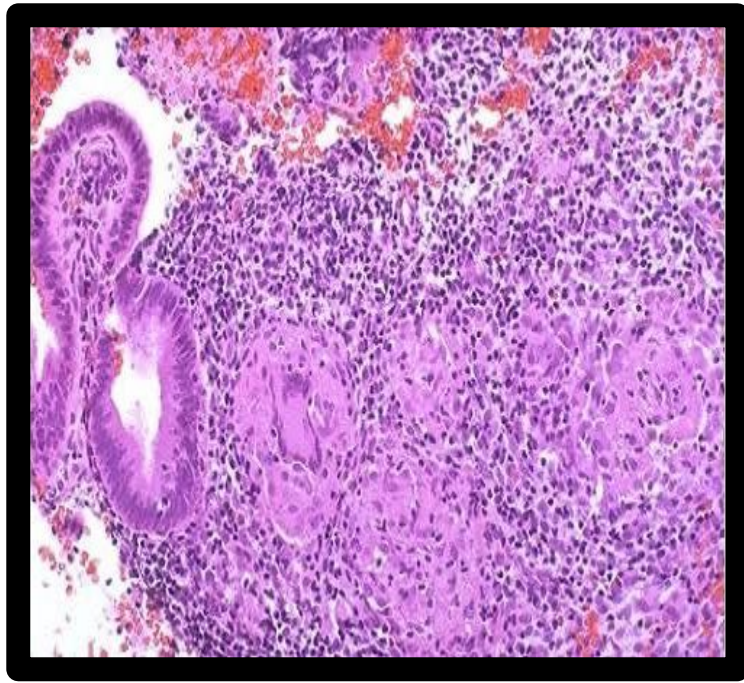
**Endometrial
Carcinoma**



**Chronic Endometritis in
Stroma**



Endometrial Polyp



**Caseating granuloma
in chronic
endometritis**

CONCLUSION

Routine Histopathological Evaluation: Essential for all AUB patients, especially those >35 years and perimenopausal women

Menstrual History Documentation: LMP correlation emphasizes the critical role of menstrual history in interpretation

Hormonal Drug History: Must be documented as it significantly influences endometrial morphology

Symptom Limitations: Clinical complaints alone cannot predict underlying pathology

Age-Based Risk Stratification: Increasing age correlates with more advanced pathological changes



REFERENCES

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- Shapley M, Jordan K, Croft PR. An epidemiological survey of symptoms of menstrual loss in the community. Br J Gen Pract. 2004; 54:359-63.
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